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/ **Practice Exam - 1**

Correct
Answer

Show

Practice Exam - 1

Question 87 of 117



A 56 year-old man with recently diagnosed ischemic cardiomyopathy presents for routine clinic visit. History is notable for anterior myocardial infarction status post PCI, diabetes mellitus type 2 (Hgb A1c 7%), hypertension, and chronic kidney disease. Recent echocardiography showed an ejection fraction of 25%. He has dyspnea walking 1 block. Medications include metoprolol succinate 150 mg daily, sacubitril-valsartan 97-103 mg bid, furosemide 80 mg twice daily, aspirin 81 mg daily, atorvastatin 80 mg daily, dapagliflozin 10 mg daily and insulin. Physical exam is notable for heart rate 62 bpm, blood pressure 110/65 mmHg, regular rhythm, 2/6 holosystolic murmur at the apex, and no signs of excess volume. Labs show creatinine 2.8 mg/dl, which is stable.

Which of the following is the next best action?

A. Start ivabradine



B. Start eplerenone



C. Refer for cardiac rehabilitation

D. Refer for nutritional assessment

Commentary

References

Testing Point: The Board-certified AHFTC specialist should prescribe cardiac rehabilitation to eligible patients to improve functional status, improve quality of life, and reduce mortality.

Answer: C. Refer to cardiac rehabilitation

Distractors: The minimally-qualified AHFTC specialist might select other answers:

Clinical trials and meta-analyses have shown that exercise training such as cardiac rehabilitation is associated with reductions in all-cause mortality and heart failure hospitalizations in patients after myocardial infarction and those with reduced ejection fraction.

Answer A: Ivabradine is recommended to reduce heart failure hospitalizations in patients with HFrEF, but it is indicated for patients with resting heart rate above 70 bpm despite a beta-blocker or those unable to tolerate a beta-blocker.

Answer B: Mineralocorticoid antagonists are recommended to reduce morbidity and mortality in patients with LVEF<35%, but patients with significant renal dysfunction (creatinine >2.5 mg/dl for men or >2.0 for women) or hyperkalemia were excluded from trials due to potential for harm. The serum potassium level is not provided.

Answer D: A nutritional assessment may be useful for education and management of heart failure, diabetes, weight loss, and malnutrition. This patient's diabetes is controlled, and no history of obesity or dietary non-adherence is mentioned.

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